# NOOR AALAM & 5 ORS v. PGIMER, CHANDIGARH VAKIL BRIEF

CC No. DC/44/CC/423/2026 · District Consumer Disputes Redressal Commission, Chandigarh
Next date: **22.12.2026 (oral arguments)** · Prepared on 30.10.2026 from the court record; updated on 31.10.2026.
The advocate should check facts and law before use. Companion files: **2. Written Arguments** (the advocate's own file, revised on the record; Word and PDF) and **3. Record File** (registers).

## 1. The case on one page

### Where the case stands

- Complainants: six legal heirs of Late Sh. Manohar, who died at PGIMER on 13.03.2026.
- Claim: Rs. 60,00,000 with 12% interest; Rs. 2,50,000 medical; Rs. 1,00,000 costs; duty doctors' details; preservation of records and CCTV.
- PGIMER denies negligence and asks for dismissal with exemplary costs.
- PGIMER's evidence closed (07.08.2026). Our rejoinder closed (14.10.2026). PGIMER's written arguments filed. **Our written arguments: 3 days from 14.10.2026 — filing not confirmed.**

### What happened (PGIMER's own records unless marked)

- **11.03.2026** — Referred from GMSH-16 for ECG changes (Wellens). PGIMER's CCU sheet: "CAD – USA (Wellen's pattern)", anaemia, plan "correct anemia"; Hb 7.5. CCU admission 10:10 am.
- **12.03.2026** — One unit of blood cross-matched and "kept ready for issue"; never issued. Moved out of the CCU onto a trolley; no ward bed. About 9:00 pm brought to the CCU once; pulse and oxygen checked; not re-admitted.
- **13.03.2026** — 12:15 am: "Received in CCU ... unresponsive & pulse was not palpable". Declared dead 12:51 am.

### Five strongest points

1. PGIMER's own diagnosis; duty admitted — OP Annexure 3 (p.15); WS para 20-G.
2. Blood planned, kept ready, never given — OP Annexures 3, 6, 7; WS para 8.
3. Out of the CCU onto a trolley — admitted — WS paras 10, 11.
4. No recorded observation from 9:00 pm to 12:15 am — OP Annexure 5 (p.17).
5. Maintainable — limitation and jurisdiction admitted (WS paras 22–23).

### Three main weak points (section 7)

1. No expert opinion yet; causation needs one.
2. PGIMER's letter: "no attendant came to collect" the blood.

3. Complaint errors to correct openly: "1:53 AM" (record: 12:51 am); "Complainant No.2" (the widow is No.3); "no monitoring after 8:30 AM"; bag "DSPRBC" (record: "SPRBC"). File 2 corrects all of these.

**Do now:** confirm whether our written arguments were filed; read the revised Written Arguments (file 2) and settle its six "[to be confirmed]" items (section 13.5); decide on an expert (section 12); decide on a records application.

**Labels** (timeline short form in brackets): PGIMER admits (Admitted: its written statement says so); Record shows (Record: a document says so); Entry (an entry exists; time or accuracy not proved); Family's account (Family: not yet proved); Does not match (two records differ); Open (needs a record, expert or family). "Our reading" = an inference to be confirmed.

## 2. Action list and contents

| Task | Who | When | Why |
|---|---|---|---|
| **Were our written arguments filed?** (3 days from 14.10.2026: 16 or 17.10.2026). If not, decide the step. | Advocate | This week | Else only PGIMER's are on file |
| Get PGIMER's written arguments (14.10.2026) from the registry; answer new points. | Advocate | This week | Not in our papers |
| **Read the revised Written Arguments (file 2)**; fill or delete its six "[to be confirmed]" items; take the decisions in section 13.5. | Advocate | Before filing | File 2 is otherwise complete |
| **Decide on a cardiologist's written opinion** (questions in section 12). | Advocate, family | Soon | Causation |
| Decide on a records application (section 10). | Advocate | Before 22.12 | Night chart, transfer note |
| Check the notice service date in the court file. | Advocate | Before 22.12 | 45-day point |
| Check each authority against the official report (section 9). | Advocate | Before 22.12 | None checked in a law report |
| Family answers (blood, bed, night, bills, income). | Family | 1 week | Facts not in the record |

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## 3. Documents map (page to cite)

Complaint bundle: cite the court page (index number); the PDF copy runs two pages ahead. WS bundle: written statement pp.1–12, annexures pp.13–25.

| Name | What it is | Filed by | Page to cite |
|---|---|---|---|
| Complaint | Complaint, memo of parties, prayer; affidavit of Complainant No.1 | Family | pp.1–35 |
| C-1 to C-5 | GMSH-16 summaries (Feb–Mar 2026); lab reports incl. NT-proBNP; ultrasound | Family | pp.36–45 |
| C-6 | GMSH-16 referral summary (10–11.03.2026) | Family | p.46 |
| C-7 | GMSH-16 blood tests and ECGs (10.03.2026) | Family | pp.47–50 |
| C-8 | PGIMER emergency note and treatment sheet | Family | pp.51–54 |
| C-9 | PGIMER Emergency BP chart | Family | p.55 |
| C-10 | Blood reaction form, slip, Rs. 150 receipt | Family | pp.56–58 |
| C-11 | Death certificate (= OP Annexure 9) | Family | index p.59; copy marked "60" — check filed set |
| WS | Written statement (dated 26.06.2026, filed 01.07.2026) | PGIMER | pp.1–12 |

| | | | |
|---|---|---|---|
| OP Ann-1 | Point-of-care print: BNP 37.4; myoglobin 176, D-dimer 3280 flagged; no name or date | PGIMER | p.13 |
| OP Ann-2, 3 | CCU sheet: chest pain, ECG; "CAD – USA (Wellen's pattern)"; plan "correct anemia" | PGIMER | pp.14–15 |
| OP Ann-4, 5 | Chest X-ray image; CCU vital chart and nurse's night note | PGIMER | pp.16, 17 |
| OP Ann-6, 7 | Hb 7.5 report; blood bank letter (01.06.2026) and register | PGIMER | pp.18, 19–20 |
| OP Ann-8, 10 | Drug chart; admission record (CCU 10:10 am; advance Rs. 2,000) | PGIMER | pp.21, 23 |
| OP Ann-11, 12 | Two ECG copies (no legible date, time or name) | PGIMER | pp.24–25 |
| Orders | 22.04, 29.05, 01.07, 07.08, 14.10.2026; e-Jagriti 22.10.2026 | Commission | by date |

## 4. Timeline, 10–13 March 2026

Every row of PGIMER's CCU chart (OP Annexure 5, p.17) is shown, with the gaps. "Entry" = time or accuracy not proved.

| Date, time | What happened | Where | Kind |
|---|---|---|---|
| 10.03, 1:07 pm | Hb 8.2 at GMSH-16 | C-7, p.47 | Record |
| 11.03, 12:14 am | Referral to PGI Emergency: ECG " (Wellens)"; troponin negative | C-6, p.46 | Record |
| 11.03, 2:04 am | PGIMER emergency registration ("GENERAL") | C-8, p.51 | Record |
| 11.03 | Emergency note: BP 130/100, pulse 116; "Imp - Wellens Syndrome"; shift to CCU | C-8, pp.52–53 | Record |
| 11.03, 5:00, 8:30 am | Emergency BP chart: 110/70; 120/88 | C-9, p.55 | Record |
| 11.03, 9:30 am | CCU chart: BP 113/73, pulse 128, respiration 20 | OP Ann-5 | Entry |
| 11.03, 10:10 am | CCU admission; advance Rs. 2,000 | OP Ann-10 | Record |
| 11.03 | CCU sheet "CAD – USA (Wellen's pattern)"; plan "correct anemia"; Hb 7.5 | OP Ann-3, 6 | Record |
| 11.03, 10:30 am | Chart row with dashes only | OP Ann-5 | Entry |
| 11.03, 12:00 pm | BP 11?/79 (digit unclear), pulse 125; 100°F, paracetamol | OP Ann-5 | Entry |
| 11.03, 3:00 pm | BP 124/103 (unclear), pulse 126, respiration 24 | OP Ann-5 | Entry |
| 11.03, 6:00 pm | BP 90/49, pulse 106, respiration 26 | OP Ann-5 | Entry |
| 11.03, 9:00 pm | BP 130/79, pulse 110, respiration 24 | OP Ann-5 | Entry |

| | | | |
|---|---|---|---|
| *— 4 hours with no entry —* | | | |
| 12.03, 1:00 am | BP 110/74, pulse 100 | OP Ann-5 | Entry |
| *— 8 hours with no entry —* | | | |
| 12.03, 9:00 am | BP 87/46; pulse "45" (unclear; could be 95) | OP Ann-5 | Entry |
| 12.03, 9:15 am | Blood request received at the blood storage centre | OP Ann-7 | Record |
| 12.03, 12:55 pm | Cross-match; unit "kept ready for issue"; bed "Trolly 66" | C-10, p.56; Ann-7 | Record |
| *— 4 hours with no entry (9:00 am to 1:00 pm) —* | | | |
| 12.03, 1:00 pm | BP 110/73, pulse 82, 37°C | OP Ann-5 | Entry |
| 12.03, 3:00 pm | BP about 93/60 (digit unclear), pulse 114; 37 or 38°C (unclear) | OP Ann-5 | Entry |
| 12.03, 4:47 pm | Rs. 150 paid for cross-matching | C-10, p.58 | Record |
| 12.03, ~5:10 pm | Out of the CCU onto a trolley (time: our reading of "nearly 31 hours") | WS PO-VI, 10–11 | Admitted |
| *— 6 hours with no entry (3:00 pm to 9:00 pm) —* | | | |
| 12.03, ~9:00 pm | Brought to CCU once: "mild breathlessness" (PGIMER) or chest pain (family); BP 90/60, pulse 82 | WS 12, 14; Ann-5 | Admitted |
| 12.03, ~9:00 pm | Family: ACC blood bank found closed | Complaint 15 | Family |
| *— 3 hours 15 minutes with no entry (on a trolley, per PGIMER) —* | | | |
| 13.03, 12:15 am | "Received in CCU ... unresponsive & pulse was not palpable"; CPR | Ann-5; WS 18 | Record |
| 13.03, 12:51 am | Declared dead | Ann-5; C-11 | Record |

## 5. The arguments

### 5.1 The complaint is maintainable

**Our point.** PGIMER charged for this admission, admits limitation and territorial jurisdiction, and does not dispute that the complainants are consumers.

**Proof.** Advance Rs. 2,000 for the CCU admission — OP Annexure 10 (p.23). Rs. 150 for cross-matching, 4:47 pm on 12.03.2026 — C-10 (p.58). Limitation and territorial jurisdiction "admitted" — WS paras 22–23 (p.11). Filed on 07.04.2026, 25 days after the death.

**PGIMER says** jurisdiction fails only because "a provable deficiency of service does not exist" (WS para 2). **Answer:** that is the merits. Paid medical service is "service" (V.P. Shantha — check current status, section 9).

**Say this in court**

"PGIMER charged Rs. 2,000 for this admission and Rs. 150 for cross-matching. It admits limitation and territorial jurisdiction (WS paras 22–23) and nowhere disputes consumer status."

**Do not say**

That Rs. 2,50,000 was paid to PGIMER — no bills on record.

### 5.2 PGIMER's own diagnosis and admitted duty

**Our point.** PGIMER recorded Wellens and "CAD – USA (Wellen's pattern)", treated acute coronary syndrome, and admits a duty of care.

**Proof.**

- Referral "regarding his recent ECG findings [CAD ?ACS?] (Wellens)"; "(Wellens)" underlined, no question mark — C-6 (p.46).
- "Imp - Wellens Syndrome"; "Shift to CCU (ACC)" — C-8 (p.52). Chest pain since 11 pm on 10.03; ECG rate 123; new T changes V2–V4 — OP Ann-2 (p.14). "CAD – USA (Wellen's pattern)", "anemia", plan "correct anemia" — OP Ann-3 (p.15).
- ACS treatment (WS para 17, p.8); "The Opposite Party owed a duty of care to the deceased" (WS para 20-G, p.10).

**PGIMER says** the complaint "grossly exaggerates" (PO-III) and the GMSH ECG showed "only '?Wellens Syndrome' ... merely suspected" (WS para 3, p.4).

**Our answer.** (1) Does not match: the question mark is on ACS, not Wellens. (2) PGIMER's own records state Wellens without a query, and it treated ACS. (3) PGIMER's own print flags D-dimer 3280 (range 0–500) and myoglobin 176 (range 0–107) as abnormal (OP Ann-1); its CCU sheet copies the normal values but not these two (OP Ann-3); no follow-up is recorded. Meaning: for the expert.

**Honest limits.** Troponin negative, BNP 37.4, normal LVEF (C-6, C-8; OP Ann-1, 2). Argue Wellens/ACS and anaemia, not heart failure.

**Say this in court**

"PGIMER's own CCU sheet records 'CAD – USA (Wellen's pattern)' and a plan to 'correct anemia'. It treated acute coronary syndrome and admits a duty of care. Its plea that Wellens was 'merely suspected' does not match the referral or its own records."

**Do not say**

"Heart failure"; "BNP ten times" (9.76 times; BNP and NT-proBNP are different tests).

### 5.3 Blood was kept ready but never given

**Our point.** PGIMER's plan was to "correct anemia" and its report showed Hb 7.5 g/dL. A unit was cross-matched and kept ready. PGIMER admits it was never issued.

**Proof.**

- Plan "correct anemia" — OP Ann-3 (p.15). Hb 7.5; same report's normal range 13.5–17.5 — OP Ann-6 (p.18).
- Request received 9:15 am on 12.03.2026; unit "kept ready for issue around 12 p.m." — OP Ann-7 (p.19).
- Cross-match 12:55 pm; transfusion boxes blank; printed rule "Get the Component issued only when the need for transfusion arises" — C-10 (p.56). Slip: "Kindly bring Blood Transport Box from the ward at the time of issue of blood" — C-10 (p.57). Rs. 150 paid at 4:47 pm — C-10 (p.58).
- No transfusion order in the drug chart — OP Ann-8 (p.21).

**PGIMER says** it was a "precautionary standby" for bleeding, Hb was "above 7 g/dL", "No medical indication" (PO-V); it was "never released or issued" (para 8); its doctor said there was "no immediate medical urgency", the trigger being "a further decline in the hemoglobin levels" (para 15).

**Our answer.** (1) "No indication" does not match PGIMER's own plan. (2) The blood bank kept the unit ready "for the patient". (3) PGIMER makes a further fall the trigger, yet no Hb after 7.5 appears in the record. (4) Its own form issues blood only when the need arises — the doctors' decision. (5) 7 g/dL is the general rule for stable patients; guidance for heart patients uses higher levels [expert to confirm; section 9]. PGIMER filed no guideline.

> **Say this in court**
> "PGIMER's own plan was 'correct anemia' and its own report shows Hb 7.5. A unit was cross-matched and kept ready, yet PGIMER admits it was never issued. Its own form issues blood only when the need arises — a decision for its doctors, who told the family there was no urgency."

> **Do not say**
> "Six hours to transfuse" (the six hours run from issue, and the six-hour words are on the identification slip, p.57; the reaction form itself says "within two hours of issue"). "The family donated two units" (the slip's "Em 26 / 2 donations" is unexplained).

**Weak spot.** "No attendant came to collect" (OP Ann-7). **Answer:** letter written on 01.06.2026, after the complaint; PGIMER's doctor said no urgency; the box comes "from the ward at the time of issue". The Rs. 150 was paid only at 4:47 pm, but PGIMER does not plead that the issue waited for payment; its case is that the unit was a standby.

### 5.4 Out of the CCU onto a trolley

**Our point.** PGIMER admits it moved him out of the CCU onto a trolley for want of a ward bed, and says the vital chart proves monitoring there. That chart has no entry after 9:00 pm.

**Proof.** WS para 11 (pp.6–7), also paras 10 and 20-b: "It is admitted that there was a temporary delay in the availability of a ward bed"; "Even on the trolley … monitoring … as conclusively evidenced by the vital chart". "nearly 31 hours" from 10:10 am (PO-VI) — our reading: about 5:10 pm on 12.03.2026. At 3:00 pm: pulse 114, BP about 93/60, first digit unclear (OP Ann-5). PGIMER's transfer rule: "coordinated between the Ward Senior Resident and the CCU Senior Resident, with the patient accompanied by hospital attendant or medical staff" (WS para 12); no transfer note or such record is produced.

**Our answer.** The shift and the trolley are admitted. We do not blame the shortage of beds; observation and an ECG needed no bed. The chart relied on stops at 9:00 pm.

> **Say this in court**
> "PGIMER admits it moved the patient out of the CCU onto a trolley because no ward bed was free. We do not blame the shortage of beds. But PGIMER says the vital chart proves he was monitored on the trolley — and that chart has no entry after 9:00 pm."

> **Do not say**
> That "Trolly 66" proves the evening trolley: the midday blood papers already show "Cardio CCU / Trolly 66" (C-10, pp.56–57). Open until the family answers and the bed register is seen.

### 5.5 The last 3 hours 15 minutes (night of 12/13 March)

**On PGIMER's own papers:** (1) out of the CCU, on a trolley (WS 10–11); (2) about 9:00 pm brought to the CCU once — "mild breathlessness" (PGIMER) or chest pain (family); pulse and oxygen checked, chest listened to; not re-admitted (WS 12, 14); (3) the chart's last entry is 9:00 pm: BP 90/60, pulse 82 — at 1:00 pm it was 110/73; (4) next: "Received in CCU at 12:15 ... unresponsive & pulse was not palpable"; dead at 12:51 am (Ann-5; WS 18).

A patient PGIMER itself diagnosed "CAD – USA (Wellen's pattern)" spent 3 hours 15 minutes on a trolley with no observation on record, and reached the CCU without a pulse.

- PGIMER gives the 9:00 pm account "to the best of our knowledge" and says saturation was "within normal limits" (WS 14). Its chart has no column for saturation; no value appears in any document produced.
- It says findings were "similar to those at initial admission" with "no tachycardia or tachypnoea" (WS 12); on the admission day its chart shows pulse 125–128 and respiration 24–26. Does not match.

- The written statement is silent on the 9:00 pm BP of 90/60. The night note mentions "ECG done" only at 12:51, with the death declaration.

> **Say this in court**
> "On PGIMER's own chart the last observation is 9:00 pm, BP 90/60. The next entry is 12:15 am: 'unresponsive and pulse was not palpable'. For a patient PGIMER had diagnosed with Wellen's pattern, no observation is recorded for three hours and fifteen minutes."

> **Do not say**
> "No monitoring at all" (say "none recorded"); "no ECG that night" (say "none of the 9:00 pm visit on record"); "no one came after 9:00 pm" (some evening drug-chart times are unclear).

**Weak spot.** The referral records "SOB x 5 days" before admission (C-6). Rely on the change on 12.03 evening, not on breathlessness alone.

### 5.6 The chart against "monitored constantly"

**Our point.** PGIMER pleads vitals "monitored constantly" and "hemodynamic stability throughout" (PO-VI; paras 15, 20-E). Its chart has 10 recorded readings over about 35 hours (the 10:30 am row is blank), including gaps of 4, 8, 4 and 6 hours, with BP 90/49 (6:00 pm 11.03), 87/46 (9:00 am 12.03) and 90/60 (9:00 pm 12.03), and pulse 125–128 on 11.03.

The drug chart's remark "vitals q_h" cannot be read on the copy (OP Ann-8). If the original reads 4- or 6-hourly, the 8-hour gap exceeds it (our reading; original needed).

> **Say this in court**
> "PGIMER's own chart has ten readings over about 35 hours, including gaps of four, eight, four and six hours, with BP readings of 90/49, 87/46 and 90/60. That is not 'monitored constantly'."

> **Do not say**
> "No monitoring after 8:30 AM" (complaint para 7). That came from the Emergency chart (C-9); withdraw it openly.

### 5.7 PGIMER's defences without documents

**Our point.** PGIMER's evidence was closed on 07.08.2026. Its medical defences rest on the written statement alone, and the documents it relies on are not produced.

- The written statement is verified "to the best of my knowledge and belief" by the Professor of Cardiology named as consultant on OP Ann-10 and C-11; no letter of authority (WS p.12).
- Not produced: committee comments (PO-II); "existing protocol" (paras 6, 10, 12); "guidelines" for Hb below 7 (paras 3, 8, 15, 20-D); counselling notes (PO-IV, 20-a);

resuscitation record (para 19); transfer note; echo and X-ray reports. OP Ann-1, 11, 12 carry no legible date or name.

- Silent on prayers (iv) and (v): duty doctors' details; preservation of records and CCTV.

> **Say this in court**
> "PGIMER's evidence was closed on 07.08.2026. It relies on committee comments, protocols, guidelines and a resuscitation finding, but has filed none of them. Its own CCU sheet, chart and blood bank letter remain — and they support us."

> **Do not say**
> That PGIMER's annexures are worthless — we rely on them. No allegation of concealment or tampering.

### 5.8 Cause of death on PGIMER's own certificate

**Proof.** Part I: (a) Type I respiratory failure, (b) aspiration pneumonia, (c) coronary artery disease; Part II: pulmonary tuberculosis — C-11 = OP Ann-9 (p.22). Emergency note "Chest – B/L AE +" (C-8, p.52); at 9:00 pm chest findings "similar to when he was admitted" (WS 14).

**PGIMER says** death was "attributable entirely to ... these underlying conditions" (PO-IV); aspiration "is based on clinical findings observed during endotracheal intubation at the time of attempted resuscitation" (para 19).

**Our answer.** PGIMER's own certificate lists coronary artery disease in Part I, the condition PGIMER itself diagnosed and was treating. Do not argue that "entirely" contradicts the certificate: PO-IV itself counts CAD among the "underlying conditions". The aspiration finding comes from intubation during CPR; no resuscitation record is filed. Whether aspiration pneumonia existed before the arrest is for the expert.

**Honest limits.** No post-mortem ("NOT WILLING FOR AUTOPSY", signed by Complainant No.1). Real earlier illnesses: TB, drug-induced hepatitis ("ATT Hepatitis" still listed on 10.03, C-6), sodium 124, fluid around both lungs (C-5).

> **Say this in court**
> "PGIMER's own certificate lists coronary artery disease in Part I. Its aspiration finding comes from intubation during resuscitation, and no resuscitation record has been filed."

> **Do not say**
> The certificate is false; the liver was normal; "1:53 AM".

### 5.9 Causation and relief

**Causation.** The sequence is PGIMER's own; whether the gaps caused or contributed to the death needs expert evidence. PGIMER says angiography was "deferred" on

clinical grounds and the family told (para 20-a); no note is filed. Our reading (expert to confirm): if anaemia was a reason, correcting it — PGIMER's own plan — was part of the path. This answers para 20-a; it is not a new allegation.

| Relief | Claimed | Proved by documents now |
|---|---|---|
| Compensation | Rs. 60,00,000 + 12% | Death and heirs; income not documented |
| Medical expenses | Rs. 2,50,000 | Paid to PGIMER: Rs. 2,000 + Rs. 150 = Rs. 2,150 |
| Litigation expenses | Rs. 1,00,000 | — |
| Doctors' details; records, CCTV | Prayers (iv), (v) | PGIMER silent |

PGIMER says the claim lacks "documentary evidence of income" (para 21), accepts three judgments as "settled law" but says they "have no application" (20-H), and asks for exemplary costs. **Answer:** the complaint rests on PGIMER's own records; keep every allegation within them.

> **Do not say**
>
> "Negligence proved" or "res ipsa loquitur" as settled; new facts outside the complaint (e.g. a shop sale) without the advocate's decision.

## 6. The 45-day point (conditional)

The order of 22.04.2026 gave "30 days from the date of receipt of notice extendable for 15 days (total 45 days)", and said that copies of the complaint, if not received, be collected from the office within 7 days of receipt of notice. PGIMER's counsel appeared on 29.05.2026. The written statement, dated 26.06.2026, was filed on 01.07.2026, 70 days after notice was ordered. It is late only if PGIMER received the notice with a copy of the complaint by 16.05.2026 (or 17.05.2026, if the day of receipt is counted); that date is not in our papers.

The advocate's file said "about 80 days". That counts from the filing of the complaint (07.04.2026) to the date on the written statement (26.06.2026); the law counts from receipt of the notice with the complaint (Hilli). File 2 therefore states the dates and keeps the point conditional, with the receipt date "[to be confirmed from the service report]".

**Two risks.** (1) On 07.08.2026 the complainants took time for a rejoinder with no objection recorded, so PGIMER may call it waiver. (2) If the written statement is shut out, PGIMER may say its annexures go with it, while our best proof comes from those annexures (CCU sheet, chart, Hb report, blood bank letter). File 2 says the complainants rely on the annexures as PGIMER's own documents. **Press the point only if the service report supports it.** Hilli was decided under s.13(2)(a) of the 1986 Act; for a service complaint the 2019 provision is s.38(3)(a) (secondary sources; official text not seen).

## 7. Weak points and answers (open ones)

| Weak point | Answer |
|---|---|
| No expert opinion | PGIMER has none either (evidence closed). Questions ready (section 12). File 2 does not claim that any step would surely have saved him. |
| No post-mortem; "NOT WILLING FOR AUTOPSY" signed by Complainant No.1 | Do not deny it. Rely on PGIMER's certificate (CAD in Part I); ask for the resuscitation record. |
| "No attendant came to collect" (OP Ann-7) | Written after the complaint; doctor said no urgency; issue only when the need arises (C-10). |
| Complaint errors: "1:53 AM"; "Complainant No.2"; "no monitoring after 8:30 AM"; "no BP" at night; "six hours" from the reaction form; bag "DSPRBC" | Correct openly: 12:51 am; widow is No.3; argue the gaps; the 9:00 pm BP is recorded; the six hours run from issue (slip, p.57); the bag reads "SPRBC" (C-10, p.56). File 2 does this. |
| Old illnesses; breathlessness before admission (C-6) | Expert to separate their share; argue the evening change and the missing 9:00 pm record. |
| Rejoinder closed; our written arguments unconfirmed | File 2 answers the written statement and denies what it does not admit (para 4). Filing date still to confirm. |
| The night account rests on Complainant No.1's affidavit, but he went home after about 9:00 pm; his mother (Complainant No.3), who stayed, has filed no affidavit | Rely on PGIMER's own chart and admissions for the night. Whether a further affidavit can be filed now is the advocate's decision. |
| Medical expenses Rs. 2,50,000: only Rs. 2,150 documented; three GMSH-16 summaries show "Billing Type: Ayushman Bharat" (C-1, C-2, C-4) | Collect the bills; claim what can be shown. PGIMER has not raised the Ayushman point. |
| Income and bills not proved | Collect documents; tie the claim to proof. |
| No written CCTV request on record | Rely on prayer (v) and PGIMER's silence. |
| Later family papers differ from the complaint | Stay with the sworn complaint unless the advocate decides otherwise. |

## 8. Likely bench questions

- **Post-mortem?** No; "NOT WILLING FOR AUTOPSY" (C-11). [Reason: ____ to be filled]
- **Where and when did he die?** Received unresponsive in the CCU at 12:15 am; dead at 12:51 am; before that on a trolley (WS 10–11, 18).
- **Why was the blood not collected?** PGIMER's doctor said no urgency (WS 15); its form issues blood only on need (C-10). [Family: ____ to be filled]
- **Expert opinion?** Not yet; PGIMER has none either. [____ to be filled]

- **Why "1:53 AM"?** The copy was misread; we accept 12:51 am. [Confirm with the family first.]
- **When were your written arguments filed?** [____ to be filled]
- **Hospitals are crowded — what could doctors do?** We do not blame the shortage; observation and an ECG needed no bed.

Final arguments are not cross-examination. Show: the orders of 01.07 and 07.08.2026; OP Ann-5; WS paras 11, 12, 14, 15, 17, 20-a; OP Ann-3 and Ann-1; C-10 and C-11.

## 9. Law and literature to verify

None checked against an official law report. On 30–31.10.2026 the official and Indian Kanoon sites were blocked by this work's network setting, so only search-engine summaries were seen (secondary). Correct citations found in secondary sources are given; confirm each before use.

| Authority | For | Check |
|---|---|---|
| CPA 2019, ss.2(7), 2(11), 2(42), 34, 35, 38, 69 | Consumer; service; jurisdiction; written version; limitation | Text in force. Secondary sources: s.38(2)(a) is for goods, s.38(3)(a) for services (file 2 keeps 38(3)(a)); s.38(9)(b) "discovery and production of any document" |
| IMA v. V.P. Shantha, (1995) 6 SCC 651 | Paid medical service is "service" | Reference declined 07.11.2024 (three judges); review and curative dismissed; a 2026 petition reported (notice, no stay) — check latest |
| Charan Singh v. Healing Touch Hospital, (2000) 7 SCC 668; AIR 2000 SC 3138 | Compensation in consumer cases | Complaint says "7 SCC 68" — correct to 668 |
| Sarla Verma v. DTC, (2009) 6 SCC 121 | Just compensation; multiplier | Complaint's "III (2009) ACC 708" is a parallel report; use SCC. A motor accident case (15.04.2009). The "bonanza" words were not confirmed; file 2 leaves them out |
| Maharaja Agrasen Hospital v. Rishabh Sharma (C.A. 6619/2016, 16.12.2019) | Medical-negligence compensation | [2019] 16 SCR 1185; 2019 INSC 1378 (secondary); no SCC citation confirmed. Para "11.5.4" not seen: file 2 states the principle without the quotation; restore it only if it matches the official copy word for word |
| Jacob Mathew (2005); Kusum Sharma (2010) | Standard of care | Citations, paragraphs |

| | | |
|---|---|---|
| Savita Garg, (2004) 8 SCC 56; Malay Kumar Ganguly, (2009) 9 SCC 221; V. Kishan Rao, (2010) 5 SCC 513 | Hospital's burden; when expert evidence is needed | Holdings |
| Paschim Banga, (1996) 4 SCC 37; Parmanand Katara, (1989) 4 SCC 286 | Resources; emergency care | Use in a consumer case |
| New India Assurance v. Hilli Multipurpose, (2020) 5 SCC 757 (04.03.2020, Constitution Bench) | 45-day limit, counted from receipt of notice with the complaint | Decided under the 1986 Act; Daddy's Builders (11.02.2021) reported to follow it; application to the 2019 Act; waiver |
| Martin F. D'Souza v. Mohd. Ishfaq, (2009) 3 SCC 1 | Referring complaints against doctors to experts | Reported (secondary) to be held per incuriam on that direction in V. Kishan Rao; read both before arguing on expert evidence |
| Bharatiya Sakshya Adhiniyam 2023, s.63; admissions | Printouts; PGIMER's statements | Text; use before the Commission |
| Effect of closed evidence; of no specific denial | Weight of the written statement | Regulations, authority |
| AABB 2023 transfusion guideline | 7 g/dL stable; 8 g/dL may be used with heart disease | Expert — literature |
| ACC/AHA 2025 ACS guideline | Hb at least 10 "may be reasonable" in ACS with anaemia (weak) | Expert |
| AABB 2025 guideline, myocardial infarction | Liberal threshold below 10 (conditional) | Expert; troponin here negative |
| StatPearls: Wellens syndrome | Normal troponin may mislead; urgent angiography | Expert — teaching review |

## 10. Records to ask PGIMER for

| Record | Why |
|---|---|
| Ward/trolley vital chart and nursing notes, 12.03 evening to 12:51 am | The 3 h 15 min |
| Transfer note; SR–SR coordination record; bed allotment register | Shift; "Trolly 66" |
| Resuscitation record and the 12:51 ECG | Aspiration; arrest |
| Original drug chart; all ECGs with machine time | "vitals q_h"; 9:00 pm |
| Duty roster and call book, night of 12.03 | Prayer (iv) |
| Blood bank issue register, hours, requisition audit log | Blood |

| | |
|---|---|
| Committee comments; protocols; counselling notes; echo and X-ray reports; verifier's authority | PGIMER's pleas |
| CCTV of 12–13.03.2026 (preservation) | Prayer (v) |
| Registry: PGIMER's written arguments; service report | Answer; 45 days |

## 11. Glossary

| Term | Meaning |
|---|---|
| Wellens pattern | ECG T-wave changes in V2–V3 linked with a tight narrowing of a major heart artery (LAD). |
| ACS; CAD; USA | Acute coronary syndrome; coronary artery disease; "USA" = unstable angina (usual shorthand). |
| CCU | Coronary care unit (heart ICU). |
| Hb | Haemoglobin; PGIMER's normal range 13.5–17.5 g/dL; his 7.5. |
| Cross-match | Test matching a donor unit to the patient before transfusion. |
| Troponin; CK-MB | Markers of heart-muscle damage; here negative or normal. |
| BNP; NT-proBNP | Two different heart-strain tests; values not comparable. |
| D-dimer; myoglobin | Markers that rise with clots or muscle injury, and with other illness. |
| LVEF | Pumping strength of the heart; "(N)" = normal. |
| MCCD | Medical certificate of cause of death. |
| PTB; ATT | Pulmonary tuberculosis; anti-TB treatment (it can injure the liver). |
| Aspiration pneumonia | Lung infection after food or fluid enters the airway. |
| SR | Senior resident doctor. |
| WS; OP | PGIMER's written statement; opposite party (PGIMER). |

## 12. Questions for a cardiologist (if the advocate decides on an opinion)

Send the expert the court pages listed in section 3, not this brief. Ask for a written opinion with reasons, the expert's qualifications, and a statement of any connection with PGIMER or the parties. For each point ask the expert to keep apart: (a) whether care fell below the expected standard, (b) what harm followed, and (c) whether that shortfall caused or contributed to the death.

1. On the records of 11–12 March, what level of care and what frequency of observation was expected?
2. How far does a negative troponin change the risk carried by a Wellens pattern?
3. With Hb 7.5 g/dL in a patient treated for ACS (with TB, and on ticagrelor, aspirin and enoxaparin), was transfusion indicated, and how urgently? What did the guidance

in force in March 2026 say, and what does "correct anemia" mean here? Was a repeat Hb expected on 12.03?

4. Angiography was "deferred" (WS para 20-a). What reasons justify deferral, and how closely should such a patient be watched? If anaemia was a reason, what was the place of transfusion?
5. Was it reasonable to move him out of the CCU on the evening of 12.03 (pulse 114 at 3 pm; the BP digit at 3 pm is unclear), and to send him back to the trolley after the 9:00 pm check? What assessment and hand-over were expected?
6. Could "mild breathlessness" in a diabetic patient under ACS treatment be an anginal equivalent? Were an ECG, troponin or re-admission expected at 9:00 pm?
7. Do the chart values (pulse 125–128; BP 90/49, 87/46, 90/60; respiration 24–26 on 11.03) show instability needing escalation? What frequency of observation would you expect?
8. Had he been observed between 9:00 pm and 12:15 am, could the arrest have been detected earlier, and with what likely difference?
9. Does the certified cause (aspiration pneumonia with coronary artery disease; TB) fit the course? Is fluid in the trachea at intubation during CPR a cause or a result of the arrest?
10. Can the share of the earlier illnesses be separated from any shortfall in care?
11. What do furosemide 40 mg then 20 mg IV twice daily and IV fluid at 30 ml/hr say about his state? Can BNP 37.4 be compared with NT-proBNP 1117.48?
12. What do OP Ann-2 (rate 123; new T changes V2–V4), Ann-11/12 and the GMSH-16 ECGs show?
13. Did the flagged D-dimer (3280) and myoglobin (176) call for further work-up?
14. Rifampicin and ticagrelor were given together (OP Ann-8). Is this relevant to his treatment or outcome?

Literature the expert may consider (the expert says whether each applies): AABB red cell transfusion guidelines 2023 (JAMA; PubMed 37824153); ACC/AHA 2025 guideline for acute coronary syndromes; AABB 2025 guideline on transfusion in acute myocardial infarction; StatPearls, "Wellens Syndrome" (NBK482490).

## 13. Written Arguments (file 2): badlav aur jaanch report

### 13.1 Kya kiya gaya

- Vakil ki apni file ("Written Arguments Noor Alam") ko hi edit kiya: wahi title block, "MOST RESPECTFULLY SHOWETH", Roman headings I–XI, "That ..." se shuru hone wale paragraph, (a)–(h) sub-para, counsel block. Paper legal size, Bookman 13 pt, double spacing, jaise vakil ki file mein.
- Har tathya court record se milaya. Jo baat sirf complaint ya affidavit par tiki hai, use "The Complainants' case is that ..." likha.
- Doosri (AI se bani) draft sirf source ke roop mein padhi; use file mein jodkar khichdi nahi banayi. File mein AI ka koi zikr nahi.
- Paragraph 47 se 51 hue (7 naye, kuch purane jode gaye taaki dohraav na ho). Word (.docx, auto-numbering) aur PDF dono.
- Jahan record se jawab nahi milta, wahan anuman nahi lagaya: chhe jagah "[to be confirmed]" hai (13.5).

### 13.2 Para-wise badlav (purana para → naya para)

| Purana | Naya | Kya badla aur kyon |
|---|---|---|
| Title | Title | Case no. khaali tha; ab "DC/44/CC/423/2026" (court orders mein yahi). |
| 1 | 1 | "alleged gross negligence" aur "critical medical condition" hataye; ab "negligence and deficiency" aur PGIMER ka apna diagnosis (CAD, anaemia). |
| 2 | 2 | s.2(7) wahi; fees ka record joda: Rs. 2,000 (OP-10, p.23), Rs. 150 (C-10, p.58). |
| — | 3 (naya) | PGIMER ne consumer hone par aapatti nahi ki; limitation aur territorial jurisdiction maane (WS 22–23). |
| — | 4 (naya) | PGIMER ke annexures ka naam "Annexure OP-1 to OP-12"; rejoinder band, isliye WS ka jawab yahin; jo maana nahi, uska inkaar. |
| 3 | 5 | Wahi; Annexure C-1, C-2, C-4, C-6 joda. |
| 4 | 6 | "indicating significant cardiac stress" hataya (record mein nahi; PGIMER ka BNP 37.4). NT-proBNP 1117.48 ke saath range 0–114.5 (C-3, p.42); Hb 8.2 (C-7, p.47); referral 12:14 AM, " (Wellens)" (C-6, p.46); WS para 3 ka apna maanna joda. |
| 5 | 7 | Doctor ka naam hataya (privacy niyam; argument par asar nahi). C-8 ki asli entries quote ki. |
| — | 8 (naya) | CCU 10:10 AM (OP-10); chest pain, ECG rate 123, V2–V4 naye T badlav (OP-2); "CAD - USA (Wellen's pattern)", anaemia, plan anaemia theek karna (OP-3); Hb 7.5 (OP-6); ACS ilaaj (WS 17); duty maani (WS 20-G). |

| | | |
|---|---|---|
| — | 9 (naya) | Case heart failure par nahi tika: LVEF normal, troponin normal (OP-1 se OP-3). Isse PGIMER ka PO-III ka hamla bekaar hota hai. |
| 6 | 10 | Saar; "chest pain" aur "third floor, staircase" ab "Complainants' case"; baaki sirf record ke tathya. |
| 7 | 11 | C-9 ki entries wahi; par maana ki C-9 Emergency ka chart hai (WS 7) aur CCU chart OP-5 hai. "8:30 ke baad monitoring nahi" wala daava waapas. |
| 8, 9 | 12, 13 | CCU chart ke asli tathya: 10 readings ~35 ghante; 4/8/4/6 ghante ke gap; BP 90/49, 87/46, 90/60; pulse 125–128; SpO2 ka column hi nahi. "elevated BNP" hataya. |
| 10 | 14 | 9 PM par PGIMER ka bayaan (WS 12, 14) aur uske saamne record; "BP nahi liya" hataya (9 PM par 90/60 likha hai); pulse oximeter wali baat "Complainants' case". |
| 11 | 15 | "OP do not dispute ... beds in the CCU" galat tha: PGIMER ne **ward** bed ki deri maani hai (WS 10, 11, 20-b). Shift ka samay ~5 PM ("nearly 31 hours" se hisaab). |
| 12, 17 | 16 | Bed ki kami ko dosh nahi; asli mudda: shift ke baad nigrani ka koi intezaam record mein nahi. |
| 13 | 17 | "stretcher, third floor, staircase" = Complainants' case; PGIMER ka "trolley"; WS 11 ka "conclusively evidenced by the vital chart" — chart mein 9 PM ke baad entry nahi; ward chart ka daava (WS 7) par chart pesh nahi. |
| 14 | 18 | "transfer summary nahi bani" → "pesh nahi ki gayi"; WS 10, 12 ke protocol wale daave quote. |
| 15, 16, 28 | 19 | "file transfer" wali baat ab ek hi jagah; "complete lack of coordination" hataya. |
| 18 | 20 | "record establishes transfusion required" → PGIMER ka plan (OP-3), Hb 7.5, requisition 9:15 AM (OP-7; slip C-10 p.57). Bag "DSPRBC" → **"SPRBC"** (C-10, p.56 par yahi likha). "Family ne blood arrange kiya" = Complainants' case. |
| 19 | 21 | "Blood Reaction Form par 6 ghante" galat tha: 6 ghante wali line slip (p.57) par hai aur **issue** se chalti hai; blood issue hua hi nahi (WS 8). Ab sawaal: blood bulaya kyun nahi gaya; form ka niyam "Get the Component issued only when the need for transfusion arises"; drug chart mein order nahi (OP-8). |
| — | 22 (naya) | PGIMER ka "standby" daava vs apna plan; "further decline" ki shart par Hb dobara jaancha nahi; guideline file nahi; payment ki shart PGIMER ne nahi kahi; expert ki jagah "[to be confirmed]". |
| 20, 21 | 23 | Complainants' case; PGIMER ka "no attendant came to collect" (OP-7) aur uska jawab: chitthi complaint ke baad (01.06.2026), Cardiology ke 29.05.2026 ke patra par; slip: box "from the ward at the time of issue of blood". |
| 22 | 24 | "No urgency" ab PGIMER ka apna maanna (WS 15). |
| 23 | 25 | Wahi, chhota. |

| | | |
|---|---|---|
| 24, 25, 27 | 26 | Complainants' case; PGIMER ka jawab (WS 12, 14, 17); jo vivaad mein nahi: ~9 PM par CCU laaye gaye, agli entry 12:15. |
| 26 | 27 | "No ECG" → "ECG record par nahi"; WS 17 ka TB wala daava aur uska jawab; OP-11/12 par date/naam nahi. |
| — | 28 (naya) | 9 PM se 12:15 AM tak 3 ghante 15 minute koi entry nahi; 12:15 "unresponsive & pulse was not palpable"; maut 12:51 AM; complaint ka "1:53 AM" ab press nahi. |
| 29 | 29 | Wahi; complaint ki "Complainant No.2" galti saaf (maa = Complainant No.3). |
| 30 | 30 | Wahi; "sirf chune hue panne aur file ka cover" joda. |
| 31 | 31 | s.38(9) wahi; do baar "That" wali galti theek; jo record pesh nahi hue unki list WS paras ke saath; prayers (iv), (v) par WS chup. |
| 32, 33 | 32, 33 | Adverse inference ab "as is permissible in law"; evidence 07.08.2026 ko band; PGIMER ke apne kagaz uske khilaaf padhe ja sakte hain. |
| 34 (a–h) | 34 (a–h) | Har item record ke hisaab se: gap ke ghante; 9 PM jaanch ka ECG/note nahi; stretcher ki jagah trolley; blood issue nahi; (g) "Complainants' case". |
| 35 | 35 | "critical condition" → "condition which it had itself diagnosed". |
| 36 | 36 | Duty PGIMER ne maani (WS 20-G); res ipsa ab seemit roop mein. |
| — | 37 (naya) | Causation: "koi ek kadam maut pakka rok deta" aisa daava nahi; MCCD Part I mein CAD; angiography "moderate anaemia" ki wajah se tali (WS 20-a); expert ki jagah "[to be confirmed]". |
| 37 | 38 | Wahi; "critically ill" ki jagah PGIMER ka apna diagnosis. |
| — | 38 ke baad | "[Reply to the written arguments of the Opposite Party ...: to be confirmed]" (copy hamare paas nahi). |
| 38 | 39 | Quotation (para 11.5.4) hataya (asli text dekha nahi gaya); principle apne shabdon mein; naam "Rishab" → "Rishabh"; para number "[to be confirmed]". |
| 39 | 40 | "(2000) 7 SCC 68" → **"(2000) 7 SCC 668"**; baaki shabd vakil ke. |
| 40 | 41 | "bonanza" wale shabd hataye (confirm nahi hue; aur ye PGIMER ke kaam ki baat thi); bataya ki ye Motor Vehicles Act ka case hai. |
| 41 | 42 | WS 20-H ka maanna joda ("settled law governing compensation in medical negligence matters"). |
| X heading | X heading | "Failure to file ..." → "Whether the written version was filed within the statutory period" (record se deri sabit nahi). |
| 42 | 43 | "approximately 80 days" hataya (galat ginti, section 6); ab tareekhen: notice 22.04, counsel 29.05, WS 26.06 dated, 01.07 filed = 70 din. |
| 43 | 44 | Wahi (s.38(3)(a)). |
| 44 | 45 | "categorically" hataya; joda ki Hilli 1986 Act ke barabar provision par tha. |

| | | |
|---|---|---|
| — | 46 (naya) | Notice milne ki tareekh "[to be confirmed]"; 16.05.2026 ya pehle mili ho tabhi deri; PGIMER ke annexures phir bhi uske apne kagaz. |
| 45 | 47 | "small clothing shop" = Complainants' case (complaint para 21). |
| 46 | 48 | "immense" hataya; baaki wahi. |
| — | 49 (naya) | PGIMER ka "unsupported by any documentary evidence" (WS 21); record par sirf Rs. 2,000 + Rs. 150; bills, income aur quantum "[to be confirmed]". |
| 47 | 50 | Wahi. |
| — | 51 (naya) | Prayer: complaint ki reliefs (Rs. 60,00,000 + 12% byaaj; Rs. 2,50,000; Rs. 1,00,000; prayers (iv), (v)). |
| Counsel block | Counsel block | Wahi; "Place:" aur "Date:" khaali; koi signature ya verification nahi. |

### 13.3 Doosri draft se kya liya, kya nahi

**Liya** (record se jaanchkar, vakil ki bhasha mein): PGIMER ka apna diagnosis aur maani hui duty (paras 8, 36); heart failure par na tikna (9); CCU chart ke gap aur values, SpO2 column na hona (12); 9 PM jaanch par WS ke daave vs record (14); trolley aur "conclusively evidenced" ka jawab (17); transfer protocol bina kagaz (18); blood: plan, Hb 7.5, "standby" daava, "further decline" ki shart, guideline nahi, Ann-7 chitthi complaint ke baad (20–23); 3 ghante 15 minute (28); jo kagaz pesh nahi hue (31); MCCD mein CAD (37); WS 20-H (42); complaint ki galtiyon ka khula sudhaar (11, 28, 29).

**Nahi liya:** "entirely" vs certificate wali dalil (PO-IV khud CAD ko "underlying conditions" mein ginta hai, isliye dalil kamzor); 3 PM ka BP "about 93/60" aur "shift se do ghante pehle" (ank saaf nahi; shift ka samay anuman); verification ke authority letter wali baat (chhoti); V.P. Shantha (verify nahi ho saka; PGIMER ne consumer status nakaara hi nahi); aspiration par lambi bahas (vakil ki file ne cause of death ko chunauti nahi di; autopsy nahi hui).

### 13.4 Kya jaancha gaya, kahan se

| Baat | Kahan se jaanchi |
|---|---|
| Req. ACC-0073148; cross-match 12/03/2026 12:55; bag 819538/B+/SPRBC/240 ml; "Trolly 66"; "Get the Component issued only when the need for transfusion arises"; "Transfuse within two hours of issue" | C-10, p.56 (page image dekha, 31.10.2026) |
| "within six hours of issue"; "Kindly bring Blood Transport Box from the ward at the time of issue of blood"; JR/SR ke sign | C-10, p.57 (page image dekha) |

| | |
|---|---|
| Referral "regarding his recent ECG findings ... (Wellens)"; "ACS" se pehle ka nishaan saaf nahi ("?" ya "with" ka chinh), isliye file sirf " (Wellens)" quote karti hai | C-6, p.46 (page image dekha) |
| "CAD - USA (Wellen's pattern)", anemia; plan "O2, correct anemia, CXR" | OP-3, p.15 (native image, zoom) |
| Raat ka note: "Received in CCU at 12:15 ... unresponsive & pulse was not palpable ... ECG done ... declared dead"; 9 PM 90/60, 82 | OP-5, p.17 (page image dekha) |
| Baaki tathya (C-3, C-7, C-8, C-9, C-11; OP-1, 2, 6, 7, 8, 9, 10, 11, 12; WS ke quote; orders) | Pichhle rounds mein har panna poora padha gaya (complaint 62/62, WS 25/25, orders); quotes un notes se milaye |
| Hisaab: 22.04→01.07 = 70 din; 07.04→26.06 = 80 din; 10:10 + 31 ghante = 5:10 PM; 9:00 PM→12:15 AM = 3 ghante 15 minute; 45 din: receipt 16.05.2026 ya pehle ho tabhi deri | Apna hisaab |
| Charan Singh, (2000) 7 SCC 668, 20.09.2000 | Secondary (search: legitquest, legalauthority, caseon, lawinsider). Shabd verify nahi |
| Maharaja Agrasen v. Master Rishabh Sharma, C.A. 6619/2016, 16.12.2019 | Secondary (vlex, order.law). Para 11.5.4 ka text NOT VERIFIED (isliye quote hataya) |
| Sarla Verma, (2009) 6 SCC 121, 15.04.2009 | Secondary. "bonanza" NOT VERIFIED (hataya) |
| Hilli, (2020) 5 SCC 757, 04.03.2020 | Secondary (manupatracademy, khaitanco, scconline blog). Shabd verify nahi |
| CPA 2019 s.2(7), s.38(3)(a), s.38(9) | Secondary only; official bare Act nahi khul saka (indiacode.nic.in network se band) |

### 13.5 Filing se pehle (Pending Issues Checklist)

1. **Filing ki tareekh:** 14.10.2026 ke order ne 3 din diye the (16/17.10.2026). Agar WA abhi tak file nahi hui, to der ke liye arzi (leave/condonation) par vakil faisla karein. File 2 par koi tareekh nahi likhi.
2. **Service report:** PGIMER ko notice + complaint copy kab mili (para 46). 16.05.2026 ke baad mili ho to Part X hata dein ya badal dein.
3. **Expert/literature:** paras 22 aur 37 ke "[to be confirmed]". Opinion na ho to placeholder hata dein; dalil bina expert ke bhi khadi hai.
4. **PGIMER ki written arguments (14.10.2026)** ki copy lein; para 38 ke baad ka placeholder bharein ya hatayein.
5. **Maharaja Agrasen:** SC ki copy mein para 11.5.4 milayein; shabd-shabd mile tabhi quote wapas daalein; para number bharein (para 39).

6. **Bills aur income** ke kagaz (para 49); na hon to placeholder hatakar claim jaisa likha hai waisa rakhein.
7. Charan Singh, Sarla Verma, Hilli aur s.2(7), s.38(3)(a), s.38(9) official text se milayein.
8. Page numbers: complaint ke index pages aur WS bundle ke apne number use kiye; Commission ki file numbering alag ho to badlein. C-11: index p.59, copy par "60".
9. Para 7 mein doctor ka naam hataya (privacy). Vakil chahein to WS para 6 / C-8 se wapas daal sakte hain.
10. Rakhne ke faisle: C-9 = Emergency chart maanna; "1:53 AM" press na karna; heart failure par na tikna. Ye record ke hisaab se sahi hain, par faisla vakil ka.
11. Part X ka khatra (section 6): PGIMER ke annexures par asar.
12. Raat ki ghatnaon ke liye Complainant No.3 ka affidavit (section 7) — vakil ka faisla.
13. Place, Date aur signature vakil bharein. File mein kisi ke "review" ya "approve" karne ki baat nahi likhi.
14. Poori file ek baar vakil khud padhein: ye unke naam se file hogi.

### 13.6 Takraav (jo aapko batana zaroori hai)

- Skill ka niyam (rule 11) kehta hai ki filing ka document draft na karein. Aapne saaf kaha ki vakil ki file edit karke do, isliye kiya; signature, verification ya tareekh nahi daali; vakil ki jaanch zaroori.
- "Koi naam, number na badlo" vs record: "DSPRBC" → "SPRBC", "Rishab" → "Rishabh", "7 SCC 68" → "668", "80 days" hataya, "6 ghante" aur "CCU bed" wali baatein sudhaari, kyunki record ya kanoon yahi kehta hai (accuracy pehle).
- Privacy: doctor ka naam hataya (badla nahi).
- s.38(2)(a) vs 38(3)(a): service ke case mein 38(3)(a) rakha (secondary sources).
- Network: indiankanoon.org aur indiacode.nic.in is kaam ke network setting mein band the, isliye kanoon official source se verify nahi hua.
- Aapne "Go to Stage 2" nahi likha tha; aapke naye sandesh ("teeno file banakar do ... is file ko editing kar kar update karna hai") ko Stage 2 ki anumati maana.